1. Emergency first aid — keeping someone alive until help arrives

Oracle OS guide. Based on IFRC 2025, European Resuscitation Council, WHO/ICRC "Basic Emergency Care" and ICRC "First aid in armed conflicts" (illustrated in the library: "IFRC International First Aid Guidelines", "US Army TC 4-02.1 First Aid", "MSF Clinical guidelines", "Hesperian Where There Is No Doctor"). Not a substitute for training — take a first aid course when you can.

Priorities

1. **Safety**: don't become another casualty (traffic, fire, electricity, gas, water, unstable structures).
2. **Call for help**: 999 or 112 (UK), 112 (Europe), 911 (US/Canada). No signal: send someone or use radio.
3. **Catastrophic bleeding** first, then **airway**, **breathing**, **circulation** (X-ABC).
4. Keep the casualty **warm**, calm and not alone. Note times and what you did.

Cardiac arrest — CPR

- Unresponsive and not breathing normally (or only gasping) = cardiac arrest. Call on speaker and send for a **defibrillator (AED)**.
- **Compressions**: centre of chest, hands stacked, arms straight, **5–6 cm** deep, **100–120 per minute**, let the chest fully recoil.
- If trained and willing: **30 compressions : 2 breaths**. Otherwise hands-only, without stopping.
- Switch the AED on as soon as it arrives and follow its voice prompts.
- **Drowning and children**: start with **5 rescue breaths**, then 30:2. Child: press 1/3 of chest depth (one or two hands). Infant: two fingers, about 4 cm.
- Continue until normal breathing returns, help takes over, or you are exhausted (swap every 2 minutes if possible).

Severe bleeding

- **Firm direct pressure** with a clean cloth for at least **10 minutes** without peeking. If it soaks through, add more on top.
- Deep wounds (groin, armpit, neck): **pack the wound** with gauze/cloth and press.
- **Tourniquet** only for life-threatening limb bleeding that pressure does not stop: 5–7 cm **above** the wound (not on a joint), tighten until bleeding stops, **write down the time**, do not release. Improvised: wide band (3–5 cm) and a stick to twist. Never thin cord.
- Lie them down and keep warm: risk of **shock**.

Shock

- Signs: pale, cold sweat, fast weak pulse, confusion, thirst.

- Lay flat, control bleeding, insulate above and below, nothing to eat or drink if surgery may be needed. Urgent help.

Choking

- Can cough → encourage coughing.
- Cannot cough, speak or breathe → **5 back blows** between the shoulder blades (lean forward), then **5 abdominal thrusts**. Alternate.
- **Infant**: 5 back blows (face down along forearm, head low) + 5 chest thrusts with two fingers. Never abdominal thrusts on infants.
- Becomes unresponsive → CPR.

Unresponsive but breathing

- **Recovery position**: on their side, top leg bent, head tilted back so the mouth drains. Check breathing every minute.
- Suspected spinal injury (fall, crash): keep the head in line; move only for danger or to open the airway.

Burns

- **Cool under cool running water for 20 minutes**, as early as possible (worth it up to 3 hours later). No ice, butter or toothpaste.
- Remove rings, watches and clothing that isn't stuck. Cover loosely with cling film or a clean plastic bag.
- Get help if: larger than the casualty's palm, face, hands, feet, genitals, joints, chemical or electrical burns, children and elderly.
- Chemicals: brush off dry powder, flush with lots of water 20+ min. Eyes: flush 15–20 min with the eyelid held open.

Fractures, sprains, dislocations

- Immobilise **in the position found**; splint across the joint above and below (sticks, magazines, cardboard, pillow). Check circulation (colour, warmth, feeling) after tying.
- Open fracture: cover with a clean cloth, don't push bone back, control bleeding.
- Sprain: rest, cold (wrapped), compression, elevation.

Cold and heat

- **Hypothermia** (shivering, confusion, slurred speech): shelter, remove wet clothes, insulate from the ground, blanket/sleeping bag, cover the head; warm sweet drinks **only** if fully alert. Severe (stops shivering, drowsy): handle very gently and horizontally.
- **Frostbite**: don't rub; rewarm in 37–39 °C water only if it will not refreeze.
- **Heat stroke** (hot, confused, may stop sweating): **cool immediately** — cold-water immersion is best; otherwise soak and fan, ice packs to neck, armpits, groin. Urgent help.
- **Heat exhaustion**: shade, lie down, water or oral rehydration solution.

Dehydration, diarrhoea, vomiting — home-made ORS (WHO)

- **1 litre safe water + 6 level teaspoons sugar + ½ level teaspoon salt.** Stir to dissolve. Should taste no saltier than tears.
- Small frequent sips, even with vomiting. Children keep breastfeeding/eating.
- Danger signs: no urine for 8+ hours, very drowsy, sunken eyes, blood in stool, baby not feeding → medical help.

Other common emergencies

- **Anaphylaxis** (swollen mouth/throat, breathing difficulty, rash): **adrenaline auto-injector** into outer thigh (through clothes), call 999/112, lie down legs raised (or sit up if breathing is hard). Second dose after 5 min if no better.
- **Asthma**: sit upright, blue reliever (salbutamol) — 1 puff every 30–60 s up to 10.
- **Heart attack** (chest pain spreading, sweating, nausea): sit, call, adult not allergic → **chew 300 mg aspirin**.
- **Stroke — FAST**: Face drooping, Arm weak, Speech slurred → Time to call; note the time.
- **Low blood sugar** (diabetic, confused, sweaty): if able to swallow, 15–20 g sugar (juice, sugar, sweets); repeat after 15 min.
- **Seizure**: clear the area, cushion the head, don't restrain or put anything in the mouth; recovery position afterwards. Over 5 minutes → help.
- **Poisoning**: don't induce vomiting; keep the packaging; burned mouth → small sips of water.
- **Snakebite** (adder in Europe): keep still, immobilise the limb, remove rings, hospital. No cutting, sucking or tourniquet.
- **Wounds**: irrigate with plenty of clean (or boiled and cooled) water, soap around it; remove dirt; cover. Infection (spreading redness, pus, fever) → antibiotics/doctor. Keep tetanus vaccination current.
- **Animal bites**: wash 15 minutes with soap and water (rabies risk); seek help.

Emergency childbirth

- Let it happen naturally; clean hands; support the baby's head, never pull.
- Dry the baby, skin-to-skin with mother, cover both. Not breathing: dry and rub the back; if still not, 5 breaths and infant CPR.
- No rush to cut the cord: wait 1–3 min, tie in two places and cut between with a clean/boiled blade. Placenta comes within 30–60 min; keep it to be checked. Heavy bleeding afterwards: rub the womb firmly through the belly and breastfeed.

Many casualties — rapid triage (START)

- **Can walk?** → GREEN (can wait).
- Not breathing after opening the airway → BLACK (dead/unsalvageable with current resources).
- Breathing over 30/min → RED (immediate).
- No radial pulse or capillary refill > 2 s → RED.

- Cannot follow simple commands → RED. Otherwise → YELLOW (delayed).
- Only stop to open airways and stop major bleeding; reassess constantly.

Minimum first aid kit

Gloves, gauze and bandages, plasters, tape, tourniquet, foil blanket, scissors, tweezers, saline, soap, paracetamol/ibuprofen, antihistamine, rehydration salts, thermometer, family's regular medication for 2+ weeks, allergy list.